



RUN DATE: [REDACTED]
RUN TIME: [REDACTED]

*** FINAL REPORT ***
SURGICAL PATHOLOGY REPORT

AGE/SEX: /M
REG DR [REDACTED]

Submit Dr: [REDACTED]

1CD-0-3

Carcinoma, papillary, diffuse sclerosing
8350/3

Site: Thyroid, nos c73.9

W
10/1/12

Addendum #1

C. RIGHT LOBE OF THYROID WITH MASS:

POSITIVE: HBME-1, p53 (rare cells), p63 (in areas of squamous metaplasia),
Cyclin D1, Galectin-3

Ki-67 proliferative index marker is up to 15%.

Addendum Signed [REDACTED]

SPECIMEN ID:

- A. LYMPH NODE - SUPERIOR MIDLINE LYMPH NODE FS,
- B. LYMPH NODE - ADDITIONAL SUPERIOR MIDLINE NODE,
- C. THYROID GLAND - RIGHT LOBE THYROID WITH MASS, D. SOFT TISSUE - PRE LARYNGEAL TISSUE,
- E. THYROID GLAND - LEFT LOBE THYROID, F. LYMPH NODE - SUSPICIOUS LYMPH NODE AT MIDLINE,
- G. SOFT TISSUE - RIGHT CENTRAL COMPARTMENT, H. SOFT TISSUE - RULE OUT PARATHYROID FS

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A. SUPERIOR MIDLINE LYMPH NODE:

METASTATIC PAPILLARY THYROID CARCINOMA to one out of one lymph node (1/1).

Metastatic deposit is 0.9 cm in greatest dimension.

B. ADDITIONAL SUPERIOR MIDLINE NODE:

METASTATIC PAPILLARY THYROID CARCINOMA to one out of one lymph node (1/1).

Minimal extranodal extension is present.

Metastatic deposit is 0.6 cm in greatest dimension.

C. RIGHT THYROIDECTOMY, 15.8 GRAMS:

PAPILLARY THYROID CARCINOMA, DIFFUSE SCLEROSING VARIANT (classical variant, tall cell variant), 3 cm in greatest dimension, with extensive lymphovascular invasion present, and extensive intrathyroidal spread.

Extrathyroidal extension is present.

Inked margins are negative for carcinoma (< 0.1 cm from nearest margin).

Tumor is present at the inked isthmus.

Adjacent thyroid with hyperplastic changes and lymphocytic thyroiditis.

No lymph nodes present.

No parathyroid gland present.

AJCC: pT3, pN1b, pMx

See Surgical Pathology Cancer Case Summary

D. PRELARYNGEAL TISSUE:

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*** FINAL DIAGNOSIS *** (Continued)

Fibroadipose tissue, negative for carcinoma.

No lymph nodes present.

E. LEFT THYROIDECTOMY, 7.4 GRAMS:

Multiple ADENOMATOUS NODULES, MULTINODULAR HYPERPLASIA, and lymphocytic thyroiditis, 0.5 cm in greatest dimension.

Inked margins are negative for carcinoma.

No lymph nodes present.

No parathyroid gland present.

F. SUSPICIOUS LYMPH NODE AT MIDLINE:

METASTATIC PAPILLARY THYROID CARCINOMA to one out of one lymph node (1/1).

Largest metastatic deposit is 0.4 cm.

G. RIGHT CENTRAL COMPARTMENT:

METASTATIC PAPILLARY THYROID CARCINOMA to six out of eight lymph nodes (6/8).

Largest metastatic deposit is 0.4 cm.

H. RULE OUT PARATHYROID:

One normocellular parathyroid gland.

Surgical Pathology Cancer Case Summary

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SPEC #: [REDACTED]

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*** FINAL DIAGNOSIS *** (Continued)

Procedure: Total thyroidectomy

Specimen Integrity: Fragmented

Specimen Size:

Right lobe: 4.5 x 3 x 1.4 cm

Left lobe: 4 x 2.5 x 1 cm

Specimen Weight:

Right: 15.8 grams

Left: 7.4 grams

Tumor Focality:

Multifocal (Ipsilateral)

Dominant Tumor:

Tumor Laterality: Right lobe

Tumor Size:

Greatest dimension: 3 cm

Additional dimensions: 2 x 1.8 cm

Histologic Type:

Papillary carcinoma:

Variant, specify:

Classical (usual)

Diffuse sclerosing variant

Tall cell variant

Architecture:

Classical (papillary)

Diffuse sclerosing

Solid

Tall cell

Cytomorphology:

Tall cell

Margins:

Margins uninvolved by carcinoma

Distance of invasive carcinoma to closest margin: 0.1 mm

Tumor Capsule: None

Lymph-Vascular Invasion: Present

Extent: Extensive (4 or more vessels)

Perineural Invasion: Not identified

Extrathyroidal Extension: Present

Extent: Minimal

Pathologic Staging (pTNM):

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*** FINAL DIAGNOSIS *** (Continued)

Primary Tumor (pT):

pT3: Tumor more than 4 cm limited to thyroid or any tumor with minimal extrathyroid extension (eg. extension to sternothyroid muscle or perithyroid soft tissues)

Regional Lymph Nodes (pN):

pN1b: Metastases to unilateral, bilateral, or contralateral cervical (Levels I, II, III, IV, V) or retropharyngeal or superior mediastinal lymph nodes Level II)

Number examined: 11

Number involved: 9

Distant Metastasis (pM):

Not applicable

Regional Lymph Nodes (pN):

pN :

Distant Metastasis (pM):

Not applicable

[REDACTED]

CLINICAL HISTORY-

PRE-OP DX: Not provided

PROCEDURE: Thyroidectomy

RELEVANT CLINICAL HX: Not provided

GROSS DESCRIPTION:

- A. Received fresh and labeled "Superior midline lymph node" is a pink-tan soft tissue fragment, 1.5 x 1.0 x 0.4 cm. Sections reveal a lymph node, 1.2 cm in greatest dimension. The lymph node is bisected and submitted in toto in one cassette for frozen section.
- B. Received in formalin and labeled "Additional superior midline" is a gray-tan lymph node, 1.3 cm in greatest dimension. The lymph node is bisected and submitted in toto in one cassette.
- C. Received in formalin and labeled "Right lobe of thyroid with mass" is a right thyroid lobe, measuring 4.5 x 3.0 x 1.4 cm and weighing 15.6 grams. No parathyroid gland is grossly identified. The isthmus surgical resection margin is inked yellow. The remainder of the surface is inked black. Sections reveal an ill-circumscribed, pale

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GROSS DESCRIPTION: (Continued)

tan, solid and firm nodule with papillary appearance, 3.0 x 2.0 x 1.8 cm. The nodule involves the mid portion and lower lobe and extends up to but not through the black-inked surface. It is 0.5 cm from the isthmus surgical margin. The thyroid parenchyma toward the upper lobe is red-brown and unremarkable. No other lesions are grossly identified. Sections are submitted as follows:

Cassettes #1&2: Upper pole
Cassettes #3-11: Mid portion with nodule (#3-10 - composite section)
Cassettes #12-14: Sections from lower pole in relation with isthmus surgical margin

D. Received in formalin and labeled "Prelaryngeal tissue" are three irregular-shaped, yellow-tan fibroadipose tissue fragments, 3.0 x 2.5 x 0.8 cm in aggregate. No lymph nodes are grossly identified. Representative sections are submitted in three cassettes.

E. Received in formalin and labeled "Left lobe thyroid" is a left thyroid, weighing 7.4 grams and measuring 4.0 x 2.5 x 1.0 cm. The isthmus surgical resection margin is inked yellow. The remainder of the surface is inked black. No parathyroid gland is grossly identified. The specimen is serially sectioned from upper to lower pole, revealing an ill-circumscribed, pale tan area located at the mid portion, 0.5 cm in greatest dimension. The remainder of the thyroid parenchyma is red-brown and unremarkable. No other lesions are grossly identified. Submitted in toto as follows:

Cassettes #1&2: Upper pole
Cassettes #3-7: Mid portion (#5-7 - pale tan area in toto)
Cassettes #8-10: Lower pole in toto

F. Received in formalin and labeled "Suspicious lymph node at midline" is a gray-tan lymph node, 0.8 x 0.8 x 0.3 cm. The specimen is bisected and submitted in toto in one cassette.

G. Received in formalin and labeled "Right central compartment" is an irregular-shaped, yellow-tan fibroadipose tissue fragment, 4.0 x 2.5 x 0.8 cm. Examination of the specimen reveals nine possible lymph nodes, from 0.3 cm to 0.7 cm in greatest dimension. The lymph nodes are submitted in toto as follows:

Cassettes #1-5: One lymph node bisected per cassette
Cassette #6: Four possible lymph nodes in toto

H. Received fresh and labeled "Rule out parathyroid" is a small fragment of pink-tan soft

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GROSS DESCRIPTION: (Continued)

tissue, 0.2 x 0.2 x 0.1 cm. Submitted in toto in one cassette for frozen section.

OPERATING ROOM CONSULT

Superior midline lymph node: Positive for carcinoma, consistent with patient's history of papillary thyroid carcinoma. Final classification however is pending permanent sections.

Rule out parathyroid: Parathyroid tissue present.

ICD CODES:

Signed (Signature on file)

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Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials		
Date Reviewed		

10/1/12